Psych Emergencies

BMRC: Nov. 2, 2026

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Scenarios

- Suspected overdose
- Acute Agitation, Paranoia, and Hallucinations
- Chest Tightness at MVC

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Psychiatric Conditions Overview

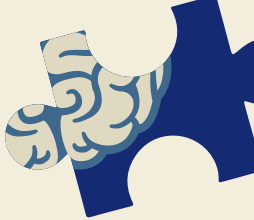
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ALCO

04

Kahoot

12YOF w/ Suspected Overdose



1. Found crying in her bedroom beside an empty medication bottle
2. Conscious, alert, complaining of nausea and dizziness
3. History of depression and suicidal ideation
4. Denies ingesting medication in front of her parents, only admits to it when alone

What is your field impression?

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Intentional Medication Overdose/ Pediatric Suicide Attempt

What is a Pediatric Suicide Attempt?

A potentially self-injurious act carried out with at least some intent to die.

Common warning signs:

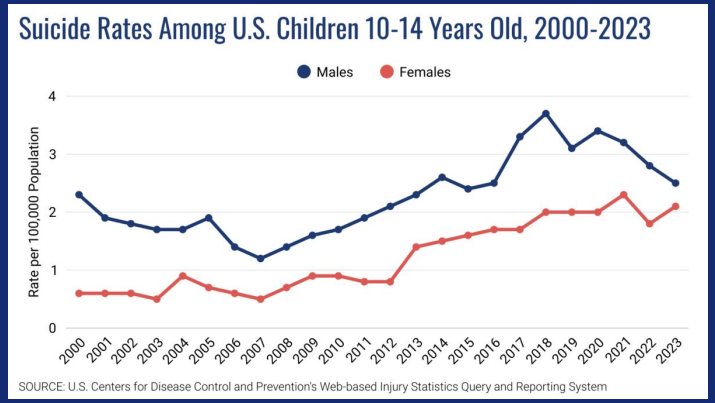
- Previous suicidal thoughts or self-harm
- Depression, anxiety, or social withdrawal
- Feelings of hopelessness or being a burden
- Sudden behavioral or mood changes
- Access to medications or other potentially lethal means

Important: A suicide attempt may occur even without a history of previous attempts or history of previous mental health concerns.

According to the CDC:

→ **20.4%** of high school students seriously considered attempting suicide in the past year.
→ **9.5%** of high school students reported at least one suicide attempt in the past year.
→ **12.6%** of female high school students reported a suicide attempt, compared with 6.4% of male students.
→ **52.6%** of female high school students experienced persistent sadness or hopelessness during the past year.

Source: *CDC, Youth Risk Behavior Survey, 2023 (published 2024)*



Pediatric Suicide

TRUE STORY!

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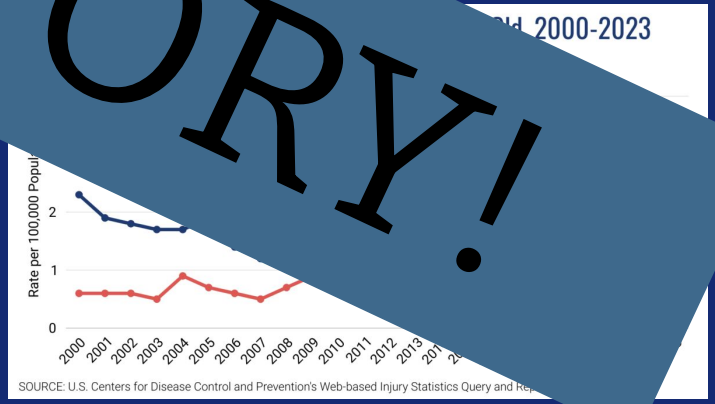
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Risk Behavior Survey, 2023 (published 2024)



Call/Text

988

Common Psychiatric Conditions in EMS



Depression & Suicidality

- Persistent sadness or hopelessness
- Withdrawal, suicidal ideation
- Possible self-harm or suicide attempts

Anxiety & Panic Disorders

- Intense fear or distress
- Rapid breathing, tachycardia
- Chest tightness, dizziness





Psychotic Disorders

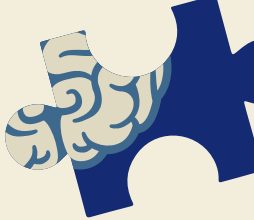
- Hallucinations or delusions
- Disorganized speech or behavior
- Possible paranoia or confusion

Bipolar Disorders

- Manic episodes: elevated mood, agitation
- Impulsivity, reduced need for sleep
- Depressive episodes



Recognizing Psychiatric Emergencies



Mental Status:

- Altered Mental Status or Disorientation
- Hallucinations (Auditory/Visual) or Delusions
- Disorganized Thought Processes or Incoherent Speech

Behavior:

- Psychomotor Agitation or Aggressive Behavior
- Social Withdrawal or Decreased Responsiveness
- Impulsivity or Behavioral Dysregulation

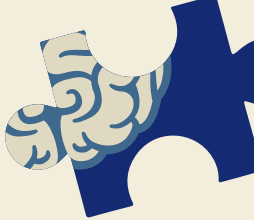
Physical Symptoms:

- Tachycardia or Tachypnea
- Tremors or Diaphoresis
- Chest Pressure or Vertigo/Lightheadedness/Dizziness

Emotional Signs:

- Extreme fear or panic
- Hopelessness or severe distress
- Suicidal or homicidal statements

Scenario 2: 28 YO Male Experiencing Acute Agitation, Paranoia, and Hallucinations



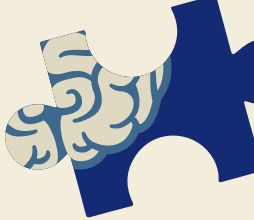
- A 28-year-old male presents with acute agitation, paranoia, and auditory hallucinations after stopping his prescribed antipsychotic medication and experiencing poor sleep.

- He is initially suspicious and uncooperative but responds to calm communication and de-escalation.

- Vital signs show mild tachycardia, and blood glucose is normal.

- He reports passive suicidal thoughts without current plan or intent.

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- Vital signs show mild tachycardia, and blood glucose is normal.
- He reports passive suicidal thoughts without current plan or intent.

Correct Impression: Schizophrenia most likely worsened by medication nonadherence.

Deciding on a 5150

What is a 5150?
An involuntary psychiatric hold for up to 72 hours for evaluation and treatment under California law.
Requirements for 5150:
Probable cause that, due to a qualifying mental health disorder, the patient meets at least one criterion: Danger to self (DTS), Danger to others (DTO), Gravely disabled (GD).

5150 vs 5250 vs 5270

Name	Duration	Purpose
5150	Up to 72 hours	Initial psychiatric evaluation
5250	Up to additional 14 days	Continued psychiatric treatment
5270	Up to additional 30 days	Extended treatment for grave disability

How to initiate a 5150:

1. Assess scene safety, ABCs, and mental status.
2. Identify and document DTS, DTO, or GD concerns.
3. Contact law enforcement or an authorized crisis professional.
4. Follow Alameda County EMS transport protocols.

Deciding on a 5150 – Special Considerations



5585: an involuntary 72-hour psychiatric evaluation and crisis stabilization hold for minors (under 18 years old)

- Specific to California!!

Some ALCO Protocols for Emergency Transport:

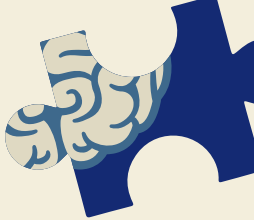
- Suspected medication overdose
- Significant intoxication or toxicity
- Altered level of consciousness or suspected shock
- Sustained HR over 120 bpm
- Blood glucose below 60 or above 250 mg/dL
- Severe agitation requiring field sedation or preventing assessment

(Among others)

Important Note: EMTs and Paramedics are not able to initiate a psych hold.

- LEOs, special mental health professionals, and mobile crisis teams are the ones who have the paperwork necessary
- If you administer ANY medical treatment, pt must be cleared at a hospital before transport to mental health facility
    - This being said, do not withhold medical treatment due to 5150 status, but consider what professionals must initiate the hold and where you dispatch them to initiate it.

Common Mental Health Condition Presentations



Depression: Persistent low mood or loss of interest
→ assess for suicidal ideation and self-harm.

Anxiety: Excessive fear or worry
→ reassure, reduce stimuli, and rule out medical causes.

Mood Disorders: Abnormal disturbances in mood
→ assess emotional state, behavior, and suicide risk.

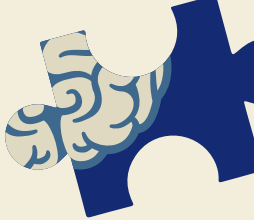
Schizophrenia: Psychotic disorder involving hallucinations, delusions, or disorganized thinking
→ avoid confrontation and use calm communication.

Bipolar Disorder: Episodes of mania/hypomania and often depression
→ assess impulsivity, agitation, and suicidality.

Paranoia: Irrational or excessive suspicion of others
→ maintain personal space and explain actions clearly.

Alzheimer's: Progressive neurodegenerative disease affecting memory and cognition
→ use simple instructions and establish baseline mental status.

Common Mental Health Condition Presentations



Dementia: Cognitive decline affecting memory, judgment, and daily functioning
→ assess for acute changes or delirium.

Dissociative Identity Disorder: Disruptions in identity and memory
→ remain supportive and assess safety.

Functional Disorders: Genuine neurological symptoms without corresponding structural disease
→ rule out acute neurological emergencies.

Organic Brain Syndromes: Cognitive or behavioral changes caused by medical or neurological conditions
→ identify and treat underlying causes.

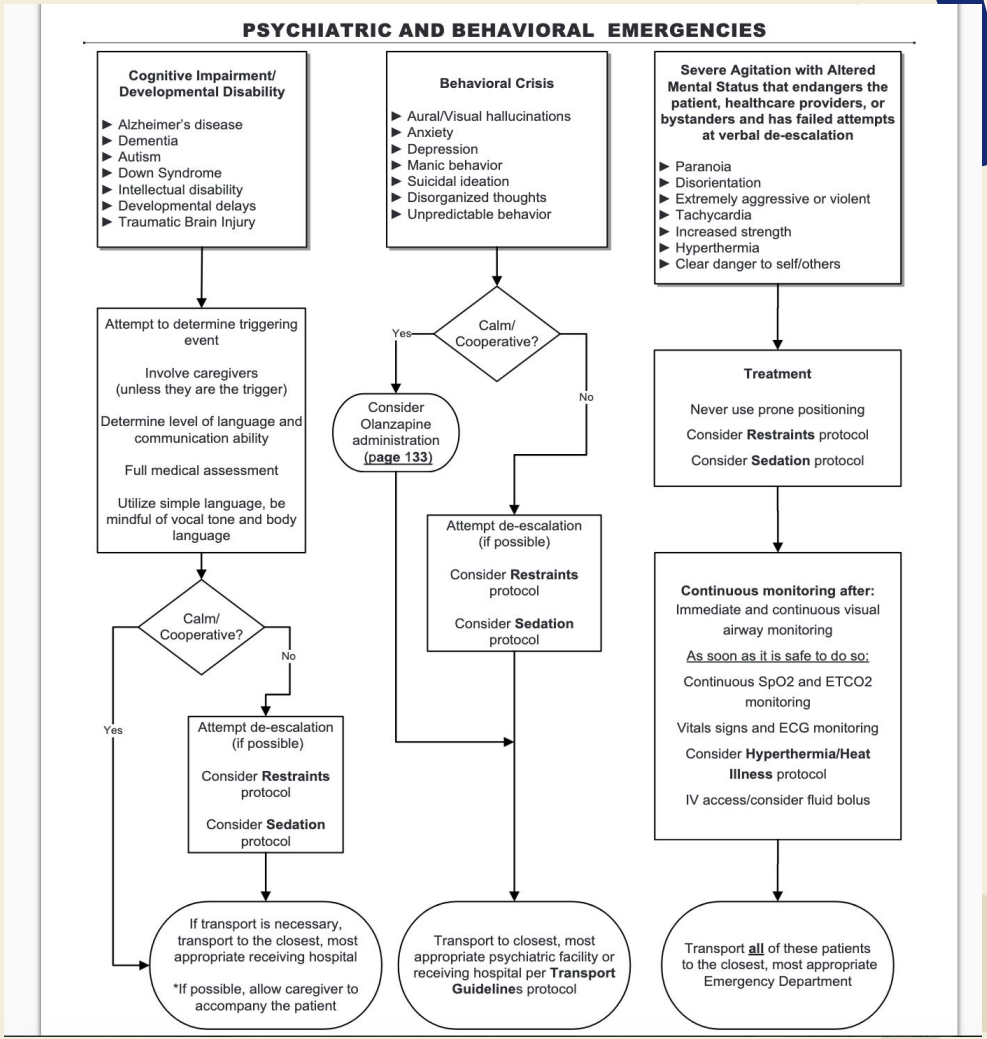
Mania: Elevated or irritable mood, increased energy, and decreased need for sleep
→ reduce stimulation and assess safety.

PTSD: Trauma-related flashbacks, avoidance, and hypervigilance
→ use trauma-informed care and minimize triggers.

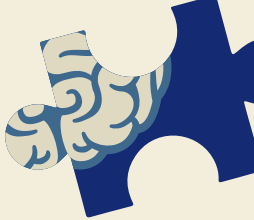
Note: Most of the time, EMTs are going to be treating symptoms of secondary medical or trauma concerns until you can get the pt to definitive psychiatric care

ALCO Protocol for Psychiatric Emergencies

- Determine whether patient has impairment, is agitated with Altered Mental Status, or is experiencing a Behavioral Crisis
- If patient is cooperative transport to facility (if needed)
- If not consider restraints and sedation



41 YO Female W/ Chest Tightness at MVC (Motor Vehicle Collision)



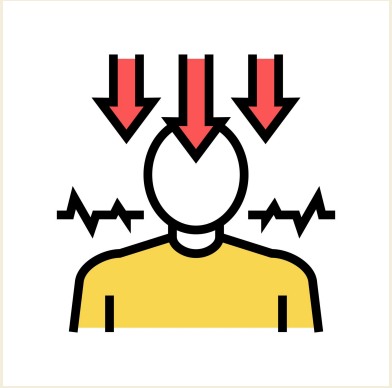
1. Bystander at a fatal head-on crash; tried to help the driver, who died before EMS arrived
2. Breathing fast, chest tightness, tingling hands and lips
3. Keeps staring at the wreck: “I should have done something.”
4. No cardiac history, but her father had an MI at 58

What is your field impression?

41 YO Female W/ Chest Tightness at MVC (Motor Vehicle Collision)

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What is your field impression?



Acute Stress Reaction w/ Hyperventilation

(Rule out cardiac and other medical causes first!)

What is an Acute Stress Reaction?

A short-term reaction to witnessing a traumatic event, such as a death. Bystanders and witnesses can be patients too.

Common warning signs:

- Rapid breathing, racing heart, shaking
- Chest tightness, tingling, lightheadedness
- Repeated thoughts or images of the event
- Strong pull toward (or away from) the scene

Important: Hyperventilation can mimic a heart attack. Never assume it is "just anxiety" without assessing.

Why do her hands tingle?

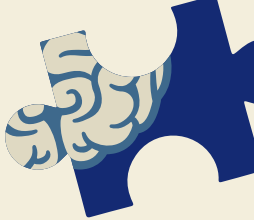
1. She breathes too fast
2. She breathes out too much CO_2
3. Her blood chemistry shifts (**respiratory alkalosis**)
4. Nerves get irritable: tingling and hand cramps

Rule these out first:

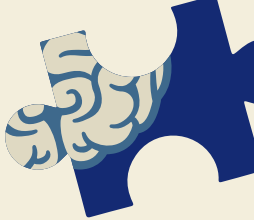
1. Heart attack
2. Low blood sugar
3. Abnormal heart rhythm
4. Asthma Attack
5. Drugs or withdrawl
6. Blood clot in lungs (PE)

Rule out Physical Causes First

- Respiratory
  - Hypoxia
  - Hypercapnia (high CO2)
  - Airway obstruction
- Neurological
  - Stroke
  - Seizures/postictal state
  - Traumatic brain injury
  - Intracranial hemorrhage
- Infectious/Environmental
  - Sepsis
  - Meningitis
  - Hypo/hyperthermia
  - Carbon monoxide poisoning
- Metabolic
  - Hypo/hyperglycemia
  - Electrolyte imbalances
  - Dehydration
- Cardiovascular
  - Hypotension
  - Shock
  - Poor cerebral perfusion
  - Cardiac arrhythmias



Rule out Physical Causes First



- Toxicological
  - Drug overdose
  - Alcohol intoxication
  - Medication toxicity
  - Substance withdrawal

Kahoot!



